

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 73 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: March 11, 2018 | Provider: Raymond Foster, MD — Cardiology | Doc ID: SMC-2018-0311-ADM

Hospital Admission — History & Physical

Acute Decompensated Heart Failure (New HFrEF)

Chief Complaint

Progressive dyspnea, leg swelling, and weight gain over 10 days.

HPI

73-year-old man with CAD (DES 2012), atrial fibrillation, T2DM, CKD G3a, treated prostate cancer presents with worsening exertional dyspnea, two-pillow orthopnea, paroxysmal nocturnal dyspnea, and 4 kg weight gain. No chest pain. Reduced exercise tolerance to <1 block. Admitted for management.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
156/96	104 irreg	24	36.7 C	91% RA	99.0 kg	32.2

Examination

JVP elevated ~12 cm. Bibasilar crackles to mid-lung fields. Irregularly irregular rhythm, S3 gallop. Pitting edema to mid-shin bilaterally. Cool extremities, peripheral pulses intact.

Admission Labs

Test	Result	Units	Reference Range	Flag
BNP	1,840	pg/mL	< 100	H
Troponin I (peak)	0.06	ng/mL	< 0.04	H
Creatinine	1.6	mg/dL	0.7 - 1.3	H
eGFR (CKD-EPI)	44	mL/min/1.73 m2	> 60	L
Potassium	4.5	mmol/L	3.5 - 5.1	
Sodium	136	mmol/L	136 - 145	
Hemoglobin	12.6	g/dL	13.5 - 17.5	L
Hemoglobin A1c	7.4	%	< 7.5	

Chest X-ray (03/11/2018)

Cardiomegaly with pulmonary vascular congestion, interstitial edema, and small bilateral pleural effusions. No focal consolidation.

Echocardiogram (03/12/2018)

LVEF 35% (newly reduced — prior 50% in 2016). Global hypokinesis with inferior/anterior wall hypokinesis. Moderate functional mitral regurgitation. LA dilated. Estimated PASP 44 mmHg.

Assessment

1. Acute decompensated heart failure, newly reduced EF (HFrEF, EF 35%) — likely ischemic + tachycardia-mediated (AF with RVR) etiology.
2. Atrial fibrillation with RVR contributing.
3. Acute-on-chronic kidney injury (cardiorenal), eGFR 44.
4. T2DM, CAD, treated prostate cancer (stable).

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Plan

- IV diuresis: furosemide 40 mg IV BID, titrate to net negative; daily weights, strict I/O.
- Rate control: continue/optimize beta-blocker; transition to carvedilol for HFrEF once euvolemic.
- Initiate guideline-directed therapy as tolerated (ACEi/ARNI, beta-blocker, MRA, SGLT2i — empagliflozin continued for HF benefit).
- Continue apixaban (renally dosed appropriate).
- Monitor renal function and potassium daily.
- Cardiology consult (Dr. Foster). Discharge planning with HF education.